
BOARD BRIEFING / HEALTHCARE TRANSFORMATION

Haven Paediatric Centre

Operational turnaround, culture, and growth

Building the culture, standards, and financial discipline that let a strong young facility grow with confidence. What the numbers show, the plan, and exactly what it costs.

For: Mr Kabir Aregbesola, Mrs Abisodun Alli, Dr Shakirah Saliu, Dr Odedina, Mr Ogochukwu Odum

From: Dr Debo Odulana, Founding Partner, Consult for Africa

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A strong young facility, ready to build for what comes next

Haven funds its own salaries, holds a steady patient base, and has earned a real reputation in Ikeja. For a facility this young, that is a genuine achievement.

Leadership is now doing what the best operators do: looking hard at the systems beneath the surface before growth outpaces them.

As Haven scales into more complex care, including neonatal intensive care, its clinical governance, its standards of work, and its financial discipline need to be deliberate rather than assumed. This engagement builds exactly that foundation, so the facility grows with confidence.

Culture is the multiplier, not a later add-on.

Process fixes rarely hold on their own. What makes safe, efficient care automatic, rather than dependent on individual diligence, is the culture, the standards of work, and the incentives beneath it. Build that foundation first, and everything above it holds.

Build the foundation first.

Then safety, margin, and growth all follow.



One root cause, showing up in three places

- **Safety.** Consistent, shift-level clinical routines are the backbone of reliable care, and formalising them is foundational as the facility grows.
- **Margin.** Margins are thin despite reasonable revenue, because no ownership culture is pushing efficiency and yield.
- **The answer is already on the table.** Two items already on the board's own decision list, the staff commission structure and the JDS and KPIs, are exactly the incentive and accountability tools a strong culture runs on.

So the work is not to patch processes. It is to build the culture, standards of work, and incentives that make good processes self-sustaining, then reengineer the processes so the culture has something solid to run on.



The cash problem is a working-capital problem, not a profit problem

Leadership feels there is “barely anything left” after salaries. The report shows why: the money is not missing. It is on the shelf and in the HMO ledgers.

~N4.2M

HMO receivables, Leadway and NEM alone, close to a full period's revenue

~N2.77M

Sitting in pharmacy stock, cash converted into inventory on the shelf

~N7M

Total working capital locked up, recoverable without selling a single extra service



NICU is the growth engine, and it rewards getting the basics right first

The highest-yield lever in the building

At an N3M deposit per admission against three beds, nothing else in the facility compares for revenue per bed. This is where growth comes from.

And what safe scaling asks first

Scaling neonatal intensive care durably means putting the governance, standards, and team readiness in place first. Get that right, and the growth compounds.

Get the governance right first. Then scale NICU with confidence, and the revenue follows.



The reporting layer is immature, and that is itself a finding

- Visit-type counts do not reconcile to total encounters.
- The receivables table does not foot.
- A 201% pharmacy “profit” is a markup, not a margin.

None of this is incompetence. It is the absence of a senior operator. You cannot manage what you cannot measure reliably, and right now the numbers the board sees cannot be fully trusted.



One board-led turnaround, five workstreams

1

Diagnostic audit (4 weeks)

Clinical governance, operations, finance and working capital, procurement, HMO economics, reporting. Two quick wins from week one: a checklist-governed crash cart, and a receivables push.

2

Culture, incentives & clinical standards of work, the spine

Shift-level routines, nursing standards of work, the safety-huddle cadence. Redesign the commission structure and JDS/KPIs around quality and ownership, not activity alone.

3

Process reengineering & operations

Vendor-managed inventory (recommend Medbury Pharma) to end stockouts and lift pharmacy margin. Build the receivables-recovery process and a reliable reporting layer.

4

Revenue & growth optimisation

Internal: NICU activation, pricing, pharmacy attach, HMO yield. External: corporate tie-ups, school partnerships (Toddler Town), a referral pipeline.

5

Board-level oversight (optional retainer)

Up to two days a month of partner-level oversight, continuing after the core project. The ongoing governance layer, offered as an opt-in.



A senior operations leader, running the facility day to day

One thing advised early still holds, and leadership is now ready for it: Haven needs a senior, experienced operator running the facility day to day.

- **Define first, then hire.** The audit specifies the role and its KPIs before anyone is recruited, so the hire lands into clarity rather than chaos.
- **Sourced through CadreHealth.** Consult for Africa's workforce platform sources and vets the candidate, including diaspora and senior-operator profiles.



Stabilise fast, build deliberately, then grow

Stabilise Weeks 1 to 2	Crash-cart standard and checklist, receivables recovery push
Diagnostic audit Weeks 1 to 4	Full audit across all functions
Culture & process build Months 2 to 4	Standards of work, incentives, procurement, reporting
Growth Months 3 to 6	NICU activation, pricing, corporate and HMO tie-ups
Oversight Ongoing (optional)	Board-level management retainer



Full value, one visible discount, then the net

Every workstream is priced at standard rates so the board sees the real value, then a single 40% partner discount line, then the net. Nothing is hidden in the rate.

Workstream	Standard	Partner (40% off)
1. Diagnostic audit (4 weeks)	N3,500,000	N2,100,000
2. Culture, incentives & standards	N5,000,000	N3,000,000
3. Process reengineering & operations	N4,000,000	N2,400,000
4. Revenue & growth optimisation	N4,500,000	N2,700,000
Core project total	N17,000,000	N10,200,000

The discount is an N6,800,000 concession, shown in full rather than buried in a lower headline rate.

Optional oversight (WS5): N1,000,000/mo standard, N600,000/mo partner, up to two days a month. Opt-in.



Cost tracks delivery, covered by the cash the early work unlocks

A mobilisation fee on signing, then the balance spread in equal monthly instalments. The receivables recovery and margin gains from the early work comfortably cover the schedule.

Payment	When	Amount
Mobilisation (diagnostic audit)	On signing	N2,100,000
Monthly instalment (x5)	Months 1 to 5	N1,620,000 each
Core project total		N10,200,000

The optional oversight retainer, if taken up, is billed monthly and separately from the schedule above.

A note on governance

Dr Odulana has been invited to Haven's board, and Consult for Africa would be a paid partner. That overlap is common and workable. Pricing and the discount are disclosed in full to all owners, the engagement is clean fixed fees plus an optional retainer, and there is no success fee tied to outcomes he would oversee. Transparency is part of the standard we are setting.



Six months from now

- A safety culture where the crash cart, and every critical routine, is checked every shift by habit, not reminder.
- The N4.2M in receivables recovered, with working-capital discipline that keeps cash moving.
- A vendor-managed inventory model that ends stockouts and lifts pharmacy margin.
- NICU admissions growing safely against a governance standard the team trusts.
- A senior operations leader in post, running a facility that measures itself honestly.
- Incentives that reward quality and ownership, so the culture sustains itself after we step back.

NEXT STEP

Commission the diagnostic audit.

If the board is content with the direction, the first step is the four-week diagnostic audit at N2,100,000 on signing. Its findings shape the detail of the culture, process, and growth work, and the timeline firms up around real data. On the go-ahead, we mobilise within the week.

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PHASE 1 OF THE ENGAGEMENT / WORKSTREAM 1

The Diagnostic Audit

Four weeks. Evidence, not impression. A costed plan of action.

The mobilisation phase of the Haven turnaround. We establish exactly where the facility stands across safety, culture, operations, and cash, and deliver two quick wins while the audit is still running.

For: Mr Kabir Aregbesola, Mrs Abisodun Alli, Dr Shakirah Saliu, Dr Odedina, Mr Ogochukwu Odum

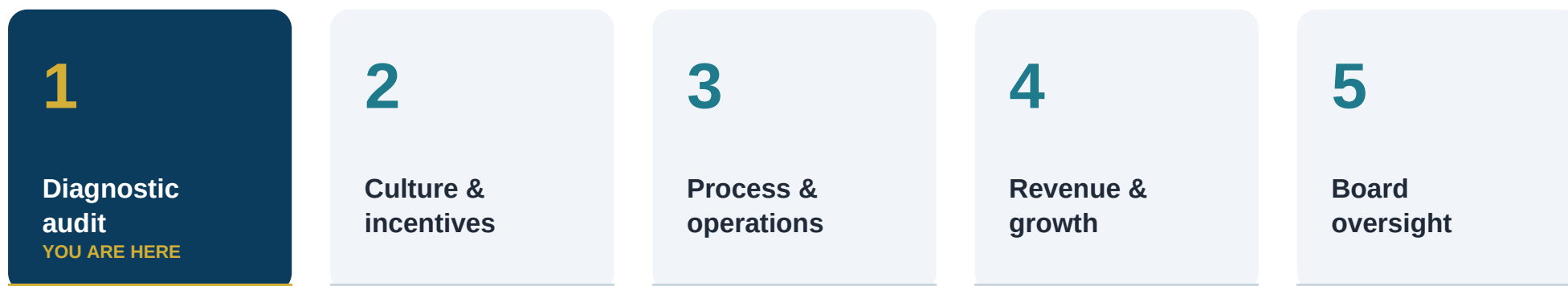
From: Dr Debo Odulana, Founding Partner, Consult for Africa

Fee: N2,100,000 on signing (standard N3,500,000, at the 40% partner discount)



One commitment now: the audit that shapes everything after it

The engagement is one board-led turnaround with five workstreams. The board only needs to commission Workstream 1 today. Its findings shape the detail, and the price, of everything that follows.



Phase 1 is the foundation. The remaining workstreams are scoped and priced off the audit's findings.



Leave the board with proof, and a prioritised, costed plan

Establish, from evidence rather than impression, exactly where Haven stands across clinical safety, culture, operations, and cash.

01 Evidence

Findings grounded in records, observation, and interviews, not in impression or blame.

02 A costed plan

Prioritised recommendations, each costed, so the board can decide what to do and in what order.

03 Value during

Two safety and cash quick wins are delivered while the audit runs, not after it reports.



Collaborative, not audit-by-ambush

- **Structured interviews.** Leadership and frontline staff, nursing, pharmacy, customer service, and operations, so the picture reflects how work actually happens.
- **Direct observation on site.** We watch the real operating day, not just the documented one.
- **Records review.** Financial, clinical, and operational records, read against each other for what reconciles and what does not.
- **A short data request in week one.** A focused list issued on day one so analysis starts immediately, not in week three.

The team is a partner in the findings, not a target of them. Honest data depends on people feeling safe to show us the real picture.



Eight areas, one connected picture

1

**Clinical governance
& patient safety**

2

**Culture, routines
& incentives**

3

**Operations &
SOP adherence**

4

**Finance &
working capital**

5

**Procurement
& inventory**

6

**HMO
economics**

7

**People &
structure**

8

**Management
reporting integrity**

The next pages go deeper on each. The areas are examined together, because the failures connect.



Clinical governance & patient safety

The question we answer: What makes acute paediatric care dependable on every shift, and ready to scale?

- **Incident and near-miss systems.** The reporting and learning process that lets the team surface and fix risks early, the marker of mature clinical governance.
- **Emergency readiness.** Crash-cart and emergency-readiness standards, and the shift-level checks that keep them dependable.
- **Clinical protocols.** Paediatric and NICU protocols, and real adherence against them.
- **NICU readiness.** What it takes to scale admissions safely, the engagement's highest-yield growth lever.

Strong clinical governance is what lets a growing facility take on more complex care, including NICU, with the team and families confident in the standard. It is the foundation everything else builds on.



Culture, routines & incentives

The question we answer: Do the culture and incentives produce safe, efficient care without anyone chasing it?

- **How work actually gets done.** The real operating day versus what the documents say.
- **Where routines break down.** The nursing and operational routines that should run every shift, and where they quietly fail.
- **Whether incentives reward the right things.** The staff commission structure and the JDS and KPIs: do they reward quality and ownership, or only activity?
- **Accountability norms.** Who owns what across shifts, and how that is held.

The commission structure and JDS/KPIs already on the board's decision list are the levers. The audit calibrates them so the culture sustains itself.



Operations & SOP adherence

The question we answer: Where does the operating model leak time, money, and goodwill?

- **Patient flow, end to end.** From arrival through care to discharge and billing.
- **Customer service and booking.** The experience that drives repeat visits and referral.
- **SOP adherence.** The gap between what is documented and what actually happens.
- **Bottlenecks.** The points that cost time, margin, or patient experience.

Thin margins on reasonable revenue point to leakage in the operating model, not to a pricing problem. We find where it leaks.



Finance, working capital & HMO economics

The question we answer: Where is the cash, and how fast can we free the ~N7M that is locked up?

- **Revenue mix and yield.** Private versus HMO, and the true margin by service line.
- **The real pharmacy margin.** Separating genuine margin from the 201% figure that is a markup, not a margin.
- **~N4.2M in receivables.** Leadway and NEM ageing and recoverability, close to a full period's revenue.
- **~N2.77M in pharmacy stock.** How much is working capital that can be released.
- **HMO contract economics.** Terms, claims, ageing, and yield per HMO versus private.

The cash problem is a working-capital problem, not a profit problem. The money is on the shelf and in the HMO ledgers, and it is recoverable.



Procurement & inventory

The question we answer: Can inventory keep every critical item available and lift margin at once?

- **Stock management.** Reorder discipline and the controls that keep critical items reliably in stock.
- **Procurement reliability.** Supplier terms, lead times, and single points of failure.
- **Margin leakage.** Where pharmacy margin is lost between purchase and sale.
- **The case for vendor-managed inventory.** Whether a VMI model, such as Medbury Pharma, keeps stock dependable and lifts margin together.

Reliable inventory does two jobs at once: critical items are always available, and pharmacy margin lifts. A vendor-managed model delivers both.



People, structure & reporting integrity

The question we answer: Is the team structured to run safely, and can the board trust its own numbers?

- **Staffing and roles.** Ratios, role clarity, and where structure creates risk or cost.
- **The senior operations leader spec.** The role defined here, so it can be recruited into clarity afterward, sourced through CadreHealth.
- **Reporting integrity.** Whether the numbers the board sees can be trusted: visit counts that do not reconcile, and a receivables table that does not foot.
- **Single source of truth.** What it takes to give the board a report it can act on.

You cannot manage what you cannot measure reliably. Fixing the reporting layer is the precondition for every other decision the board makes.



Two quick wins, delivered during the audit

The board sees safety and cash improve in week one, before any findings are written up.

1. A crash cart you can trust

Stocked, sealed, and checklist-governed, with a shift-level check routine, live in week one. A core safety standard put in place from the start, not deferred to the end.

2. Cash starts moving

An immediate recovery push on the largest HMO receivables, Leadway and NEM, so money starts coming in before the audit even reports. The work begins to pay for itself.



Two deliverables, presented to the board

Both are walked through with the board in a working session at the end of week four, not emailed as a document to file away.

1. Operational & Clinical Governance Audit Report

Findings across every area examined, each with a prioritised, costed recommendation, so the board can act in order of impact and decide the shape of the workstreams that follow.

2. Working Capital & Receivables Recovery Plan

A concrete plan to unlock the cash tied up in the ~N4.2M of receivables and ~N2.77M of pharmacy stock, with the routines that keep it from locking up again.



Mobilise in week one, present in week four

Week 1

Mobilise

Stakeholder interviews, data request issued, crash-cart standard live, receivables push begins

Week 2

Care & operations

Clinical governance and safety, culture and routines, operations and SOP walkthrough

Week 3

Money & numbers

Finance and working capital, procurement and inventory, HMO economics, reporting integrity

Week 4

Synthesis

Prioritised, costed recommendations, and the board presentation



A light lift on your side, set up in week one

- **A named point person.** We suggest the Head of Operations, as the single channel for the engagement.
- **Read access to the records.** Revenue, receivables ageing and P&L, pharmacy stock data, HMO contracts, the staff roster and job descriptions, clinical incident records, and existing SOPs.
- **Time with the people who do the work.** Leadership, plus the nursing, pharmacy, and customer-service leads.
- **Walk-through access to the facility.** So observation reflects the real operating day.

NEXT STEP

Commission the audit. We mobilise within the week.

The diagnostic audit is N2,100,000, payable on signing (standard rate N3,500,000, at the 40% partner discount). On the board's go-ahead, we issue the data request, set the crash-cart standard live, and begin the receivables push, all in week one.

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Haven Paediatric Centre

Proposal for operational turnaround, culture, and growth

Building the culture, standards of work, and incentives that make good care self-sustaining. Then reengineering the operations and unlocking growth.

Date: 27 June 2026

For: Mr Kabir Aregbesola, Mrs Abisodun Alli, Dr Shakirah Saliu, Dr Odedina, Mr Ogochukwu Odum

From: Dr Debo Odulana, Founding Partner, Consult for Africa



1. Why we are having this conversation

Haven Paediatric Centre was built to raise the standard of paediatric care in Nigeria. Fifteen months in, it is funding its own salaries, holding a steady patient base, and earning a real reputation in Ikeja. That is a genuine achievement for a young facility.

Leadership is now doing what the strongest operators do: looking hard at the systems beneath the surface before growth outpaces them. As Haven scales into more complex care, including neonatal intensive care, its clinical governance, its standards of work, and its financial discipline need to be deliberate rather than assumed. Putting that foundation in place now, while the facility is still young, is far easier than retrofitting it later.

This proposal sets out how Consult for Africa would help, the shape of my own involvement, and exactly what it costs, at full transparency.

2. The honest diagnosis: culture is the multiplier

It is tempting to treat operational improvement as a set of process fixes. In practice they rarely hold unless the culture, the standards of work, and the incentives beneath them are built first. Strong routines are what make safety and efficiency automatic, rather than dependent on individual diligence.

The same theme shows up across the business:

- Consistent, shift-level clinical routines are the backbone of reliable care, and formalising them is foundational as the facility takes on more complex cases.
- Margins are thin despite reasonable revenue, because there is no ownership culture pushing efficiency and yield.
- Two items already on the board's own decision list, the staff commission structure and the JDS and KPIs, are exactly the incentive and accountability tools a strong culture runs on. The report is already pointing at the answer.

So the work is not to patch processes. It is to build the culture, the standards of work, and the incentives that make good processes self-sustaining, then reengineer the processes themselves so the culture has something solid to run on.

3. What the last management report already tells us

The cash problem is a working capital problem, not a profit problem.

Leadership feels there is "barely anything left" after salaries. The report shows why: roughly N4.2M sits in HMO receivables (Leadway and NEM alone are the whole balance), close to a full period's revenue, and a further N2.77M sits in pharmacy stock. That is around N7M of working capital locked up. The money is not missing. It is on the shelf and in the HMO ledgers.

NICU is the growth engine, and it rewards getting the basics right first.

At a N3M deposit per admission against three beds, NICU is the single highest-yield lever in the building. Scaling those admissions durably depends on putting the governance, standards, and team readiness in place first. Get that right, and the growth compounds. That is the through-line of the whole engagement.



The reporting layer is immature, and that is itself a finding.

The visit-type counts do not reconcile to total encounters, the receivables table does not foot, and a 201 percent pharmacy “profit” is actually a markup, not a margin. None of this is incompetence. It is the absence of a senior operator. You cannot manage what you cannot measure reliably.

4. Proposed involvement and engagement structure

A single board-led turnaround with five workstreams. The first four are the core project. The fifth is an optional, ongoing oversight retainer leadership can opt into.

1. Diagnostic audit (4 weeks)

A detailed audit across clinical governance, operations and SOP adherence, finance and working capital, procurement and inventory, HMO economics, staffing, and the reporting layer. Two quick wins run from week one: a checklist-governed crash cart, and an immediate recovery push on the Leadway and NEM receivables.

2. Culture, incentives, and clinical standards of work

The spine of the engagement. Establish the shift-level clinical routines, the nursing standards of work, and the safety-huddle cadence. Redesign incentives: the commission structure and the JDS and KPIs already awaiting board approval, aligned to quality and ownership rather than activity alone.

3. Process reengineering and operations

Move procurement and inventory to a vendor-managed model. My specific recommendation is to engage Medbury Pharma for vendor-managed inventory, which keeps critical stock dependable and lifts pharmacy margin at the same time. Build the receivables recovery process and a reliable management reporting layer.

4. Revenue and growth optimisation

Internal growth (NICU activation, pricing review, pharmacy attach, HMO yield) and external growth (corporate tie-ups, school partnerships such as Toddler Town, and a referral pipeline).

5. Board-level management oversight (optional retainer)

Up to two days per month of partner-level oversight from me or a delegate, continuing after the core project. The ongoing governance layer, offered as an opt-in.

5. The senior operations leader

One thing I advised early still holds, and leadership is now ready for it: Haven needs a senior, experienced operations leader running the facility day to day. The audit defines that role and its KPIs first, so the hire lands into clarity rather than chaos. Consult for Africa can source and vet that person through CadreHealth, our workforce platform, including diaspora and senior operator profiles. Define the role during the engagement, then recruit into it.



6. Timeline

Phase	Timing	Focus
Stabilise	Weeks 1 to 2	Crash-cart standard and checklist, receivables recovery push
Diagnostic audit	Weeks 1 to 4	Full audit across all functions
Culture and process build	Months 2 to 4	Standards of work, incentives, procurement, reporting
Growth	Months 3 to 6	NICU activation, pricing, corporate and HMO tie-ups
Oversight	Ongoing (optional)	Board-level management retainer

7. Commercials, in full transparency

We price every workstream at standard Consult for Africa rates so the board sees the real value, then show a single partner discount line, then the net. Nothing is hidden in the rate. Given our longstanding relationship, and that I have been invited to Haven's board, the facility receives a 40 percent discount on every line.

Workstream	Standard rate	Partner rate (40% off)
1. Diagnostic audit (4 weeks)	N3,500,000	N2,100,000
2. Culture, incentives and standards	N5,000,000	N3,000,000
3. Process reengineering and operations	N4,000,000	N2,400,000
4. Revenue and growth optimisation	N4,500,000	N2,700,000
Core project total	N17,000,000	N10,200,000

The discount is a N6,800,000 concession, shown in full rather than buried in a lower headline rate.

Optional ongoing oversight (Workstream 5): standard N1,000,000 per month, partner rate **N600,000 per month**, for up to two days per month of partner-level oversight. Priced to senior-partner value and offered as an opt-in alongside the core project.

Payment schedule: a mobilisation fee on signing, then the balance spread in equal monthly instalments, so cost tracks delivery and is comfortably covered by the receivables and margin the early work unlocks.

Payment	When	Amount
Mobilisation (diagnostic audit)	On signing	N2,100,000
Monthly instalment (x5)	Months 1 to 5	N1,620,000 each
Core project total		N10,200,000



The optional oversight retainer, if taken up, is billed monthly and separately from the schedule above.

8. A note on governance

I have been invited to Haven's board, and Consult for Africa would be a paid partner to the company. That overlap is common and entirely workable, and I am raising it precisely because formalising your governance is part of this work. The pricing and the discount are disclosed in full to all owners, and the engagement is structured as clean fixed fees plus an optional retainer, with no success fee tied to outcomes I would oversee. Transparency here is part of the standard we are setting for the facility.

9. What good looks like in six months

- A safety culture in which the crash cart, and every critical routine, is checked every shift, by habit, not by reminder.
- The N4.2M in receivables recovered and a working-capital discipline that keeps cash moving.
- A vendor-managed inventory model that ends stockouts and lifts pharmacy margin.
- NICU admissions growing safely against a governance standard the team trusts.
- A senior operations leader in post, running a facility that measures itself honestly.
- Incentives that reward quality and ownership, so the culture sustains itself after we step back.

10. Next steps

If the board is content with the direction, the first step is to commission the diagnostic audit. From there, the audit findings shape the detail of the culture, process and growth work, and the timeline firms up around real data.

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